

****HOSPITAL DISCHARGE SUMMARY****

****PATIENT INFORMATION****

Name: Patient 19

Age: 70 years old | Sex: Male

MRN: 000-TEST-19

Admission Date: 06/12/2026 | Discharge Date: 06/19/2026

Facility: Metro Medical Center

****PRIMARY DIAGNOSIS****

Acute exacerbation of Chronic Obstructive Pulmonary Disease (COPD), GOLD Stage II (mild-to-moderate airflow limitation)

****SECONDARY DIAGNOSES****

- Chronic gout, currently controlled
- Benign Prostatic Hyperplasia (BPH) with nocturia
- Hypertension
- Hyperlipidemia

****HOSPITAL COURSE****

Patient 19 presented to the ED with acute dyspnea, productive cough (green sputum), and decreased exercise tolerance x 3 days. Baseline FEV1/FVC ratio 58% (moderate obstruction). Initial SpO2 on room air 87%, improved to 92–94% with supplemental oxygen at 2L via nasal cannula. Chest X-ray showed mild hyperinflation without acute infiltrate. Patient was treated with IV methylprednisolone 125 mg q.i.d., continuous nebulized albuterol/ipratropium alternating q.2h, and IV ceftriaxone 1g q.12h for presumed infectious trigger (viral vs. atypical bacterial etiology). ABG on admission: pH 7.34, pCO2 48, pO2 62

(supplemented). Respiratory status improved steadily; transitioned to inhaled therapy by hospital day 2. Patient tolerated increased ambulation by discharge; SpO2 maintained 94–96% on room air at rest, 88–92% with ambulation. No acute complications. Discharged on optimized inhaler regimen with systemic corticosteroid taper.

****DISCHARGE MEDICATIONS****

| Medication | Dose | Frequency | Duration |

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| Tiotropium (LAMA) | 18 mcg | Once daily | Indefinite |

| Salmeterol/Fluticasone (LABA/ICS) | 50/250 mcg | BID (morning & evening) | Indefinite |

| Albuterol rescue inhaler | 90 mcg/actuation | PRN (q.4–6h) | Ongoing |

| Prednisone taper | 40 mg days 1–3, 30 mg days 4–5, 20 mg days 6–7, 10 mg days 8–9, 5 mg day 10 | As scheduled | 10-day taper |

| Allopurinol | 300 mg | Once daily | Indefinite |

| Finasteride | 5 mg | Once daily | Indefinite |

| Lisinopril | 10 mg | Once daily | Indefinite |

| Atorvastatin | 40 mg | Once daily at bedtime | Indefinite |

****ACTIVITY & RESTRICTIONS****

No heavy lifting or strenuous exertion x 2 weeks. Resume normal ADLs as tolerated. Gradual increase in walking distance; target 15–20 minutes daily if tolerated. Avoid crowds and respiratory irritants; use mask in public spaces if feeling unwell.

****DIETARY RECOMMENDATIONS****

Low-sodium diet (< 2g daily); adequate fluid intake (1.5–2L daily unless otherwise restricted). Avoid foods high in purine due to gout history (limit red meat, organ meats, high-fructose

items).

****FOLLOW-UP APPOINTMENTS****

1. ****Pulmonology clinic**** —